

Our Ref: (T) TS/AS/ [MED-ID]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Joint IBD Clinic

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

Room CG89, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME]

[ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Ileal Crohn's disease diagnosed on capsule endoscopy

Intermittent small bowel bacterial overgrowth

Chronic malnutrition

Osteoporosis

Pancreas divisum with previous pancreatic sphincterotomy

Previous intolerance to budesonide and prednisolone

Hyperparathyroidism with surgical parathyroidectomy 2015

I reviewed Gina in the GI Clinic today. [NAME] was in good spirits and relatively well which down to the warm summer weather.

When I last saw her I suggested rotating her antibiotics to treat presumed bacterial overgrowth had an episode of angioedema with Metronidazole which [NAME] obviously will now need to avoid a course of Rifaximin in April which was helpful although [NAME] finds the treatment difficult to tolerate at the time. I have suggested that if [NAME] has recurrent symptoms we could try Cip in the first instance followed by Rifaximin and I have given her a hospital script for these [NAME] has a supply at home. [NAME] can phone us directly for advice if [NAME] needs some guidance [NAME].

[NAME] has found the support of our specialist dietitian very helpful and [NAME] is having 10g of Elemental 028 a day. [NAME] is just about maintaining her weight 39.6kg today.

[NAME] was given a trial of Creon on the advice of my dietetic colleagues which I thought was reasonable. [NAME] felt unwell with one dose of Creon but I am not sure [NAME] represents a reaction to Creon. [NAME] is open-minded about trying [NAME] again in the future but wants to avoid any medical interventions over the summer months. The only other thought that Gina herself came up with is that the bile salt handling issues could be contributing to her gut symptoms. [NAME] is a good thought. We have booked a SeHCAT scan in the autumn to look for [NAME].

We will see her again in three to four months' time.

With best wishes

Yours sincerely

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[PROVIDER-NAME]

Consultant Gastroenterologist

Copy to:

[NAME]

[NAME]

[NAME]